Pacific Spine & Orthopedics

900 Pacific Ave, Floor 2, Seaview, CA 90000 (fictional)

NEW PATIENT CONSULTATION

Patient: Marisol Vega **DOB:** 07/19/1988

Date of visit: 02/24/2026 **Provider:** Helen Park, MD

History

Referred after a motor vehicle collision on 02/03/2026. Neck pain 6/10 with intermittent pain and tingling into the left forearm. Low back pain improving. MRI of 02/17/2026 reviewed.

Examination

Spurling test positive on the left. Reduced cervical rotation. Strength 5/5. Reflexes symmetric.

Assessment

1. M50.222 - Cervical disc displacement at C5-C6 level
2. S13.4XXA - Sprain of ligaments of cervical spine

Plan

Physical therapy two times a week for six weeks. Continue ibuprofen as needed. Return in 6 to 8 weeks.

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FOLLOW-UP VISIT

Patient: Marisol Vega **DOB:** 07/19/1988

Date of visit: 04/21/2026 **Provider:** Helen Park, MD

Interval history

Completed six physical therapy sessions, then stopped after 03/19/2026 because of her work schedule. Neck pain persists at 6/10 with left arm radiation that worsens with overhead work.

Assessment

M50.222 - Cervical disc displacement at C5-C6 level, persistent radicular symptoms despite conservative care.

Plan

Cervical interlaminar epidural steroid injection at C5-C6. Risks, benefits and alternatives discussed; patient agrees.

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PROCEDURE NOTE

Patient: Marisol Vega **DOB:** 07/19/1988

Date of procedure: 05/05/2026 **Physician:** Helen Park, MD

Procedure: Cervical interlaminar epidural steroid injection, C5-C6, under fluoroscopic guidance (CPT 62321, 77003).

Pre-procedure diagnosis: M50.222 - Cervical disc displacement at C5-C6 level.

Description: Prone position, sterile preparation. Under fluoroscopy, the C5-C6 interlaminar space was entered using a loss-of-resistance technique. Contrast confirmed epidural spread. Injected 80 mg triamcinolone with 3 mL preservative-free saline. No complications.

Pain: 6/10 before, 2/10 after the procedure.